Project Larkspur

Confidential Information Memorandum

December 2026

Prepared by Falkmere Advisory, exclusive financial advisor to Orivane Clinical Laboratories, LLC

Orivane Clinical Laboratories: accurate, timely diagnostics for community physicians, care operators and their patients across the Northeast

STRICTLY PRIVATE & CONFIDENTIAL

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Executive Summary

A scalable, accredited regional reference laboratory with 21.4% adjusted EBITDA margins and three funded growth engines

- Platform
  - Founded in 2009 by Dr. Samuel Achterberg, Orivane Clinical Laboratories operates a 24,000 sq. ft. centralised core laboratory in State A, 6 patient service centres and a company-owned courier fleet serving physician practices, urgent-care clinics, behavioural-health providers and senior-living communities across the Northeast region.
  - Single-site model with an LIS/EMR-integrated workflow: 268 working days of 379 average daily accessions in 2026E, 12-24 hrs routine turnaround and 99.6% first-pass reporting accuracy.
- Clinical excellence
  - National laboratory licence and independent accreditation-body accreditation maintained without deficiency since 2016; 410 unique tests across routine chemistry and haematology, toxicology, molecular/PCR and anatomic-pathology send-outs, plus 14 custom panels built for client specialties.
  - Board-certified laboratory and medical directors, a 41-person bench of technologists and net logo retention of 103% across the trailing twelve months.
- Compelling market characteristics
  - National clinical laboratory testing spend of $91.7 billion in 2025 is projected to grow to $135.7 billion by 2035, driven by chronic-condition prevalence, an ageing population and the shift of hospital outreach volume to independent labs.
  - Balanced reimbursement across government programmes (43% of net revenue) and seven in-network commercial payers, none above 17%.
- Strong financial profile
  - Net operating revenue of $15.0M and adjusted EBITDA of $3.2M (21.4% margin) in 2026E, an 8.5% revenue CAGR since 2024A with margins expanding from 19.5% to 21.4%.
  - Management projects $18.9M of revenue and $4.5M of adjusted EBITDA by 2028E, driven by the Silvermoss Senior Living contract, two sales representatives starting in March 2027 and utilisation of existing capacity.
- Highly experienced management team
  - A six-person leadership team averaging 13 years with the Company, spanning laboratory science, revenue-cycle finance, commercial development and logistics, with a documented succession plan for the Medical Director role.

Company snapshot	
Founded	2009 (Dr. Samuel Achterberg)
Headquarters	Core laboratory - State A (Northeast region)
Services	Routine chemistry & haematology 71%; Molecular / PCR 19%; Anatomic pathology send-outs 6%; Toxicology 4% of 2026E net revenue (1)
Payers	Government programmes 43%; commercial 57% (1)
Clients	Physician practices & IPAs 57%; Behavioural-health providers 12%; Urgent-care clinics 13%; Senior-living communities 8%
Ownership	Founder 70%; two long-tenured executives 15% each (LLC membership interests)
Employees	95 (payroll employees; couriers and per-diem phlebotomists included) (2)

Certifications & accreditations

National laboratory licence	**Independent accreditation-body accreditation**	**State A licence**
Multi-state licences (7)	**Payer credentialing**	

(1) Mix based on 2026E net operating revenue. (2) Headcount at 30 Nov 2026. (3) Market data: Industry Research Firm A (synthetic). Source: Company management; QoE report (QoE Advisor) through Aug-26; Falkmere analysis.

Company by the Numbers

Core operating and financial metrics validate a scaled, efficient and accurate laboratory platform

$15.0M 2026E net operating revenue	**$3.2M** 2026E adjusted EBITDA (21.4% margin)	**95** Employees (1)	**101,458** 2026E accessions
14 Custom diagnostic profiles	**410** Unique tests on menu	**24,000** Core laboratory sq. ft.	**6** Patient service centres
62 Analytical instruments	**12-24 hrs** Routine turnaround time	**99.6%** First-pass reporting accuracy	**520** Max daily accessions, single shift
103% Net logo retention, LTM Nov-26	**73%** 2026E utilisation of single-shift capacity	**42.5%** 2026E collection rate (net / gross charges)	**338** Unique ordering clients, YTD Nov-26

The laboratory processed 100,731 accessions in the twelve months to November 2026 at an implied net reimbursement of $147.38 per accession, generating $14.8M of LTM net operating revenue. Average daily throughput of 379 accessions in 2026E represents 73% of single-shift capacity, leaving room to absorb the contracted senior-living volume and the 2027 sales-team build before a second shift is required.

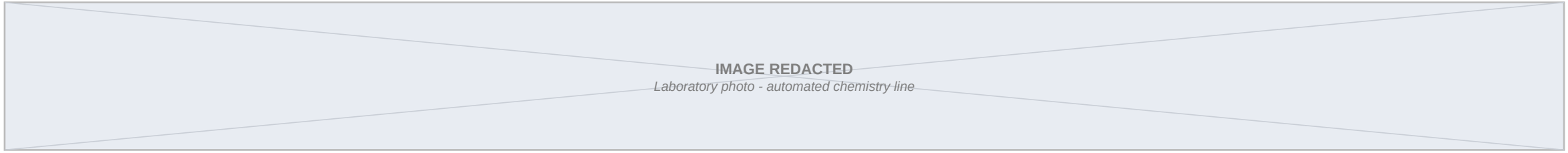
(1) Employees at 30 Nov 2026, including per-diem phlebotomists and couriers on payroll; excludes 4 contracted pathologists. Source: Company management; QoE report (QoE Advisor) through Aug-26; Falkmere analysis.

History and Corporate Timeline

Seventeen years of disciplined, self-funded growth from a boutique chemistry lab to a full-menu regional reference laboratory

2009	2012	2015	2017	2019-20	2022	2024	2025	2026-28
Founded by Dr. Samuel Achterberg as a specialty chemistry laboratory serving 30 physician practices	Licensed in four additional Northeast states; first dedicated courier routes and second draw site opened	Toxicology and molecular departments launched; first automated chemistry line installed	Relocation to the current 24,000 sq. ft. core laboratory; Dr. Oyelaran appointed Laboratory Director	Second molecular suite commissioned and pharmacogenomic panels validated, funded from operating cash flow	LIS upgrade, outsourced revenue-cycle platform and quality system rebuild; PSC network reaches 6 sites	$12.7M of net revenue at a 19.5% adjusted EBITDA margin; senior-living channel strategy defined	Silvermoss Senior Living contract live from April; revenue up 8.2% to $13.8M	Re-accreditation; QoE completed through Aug-26; plan to reach $18.9M by 2028E with new reps from Mar-27

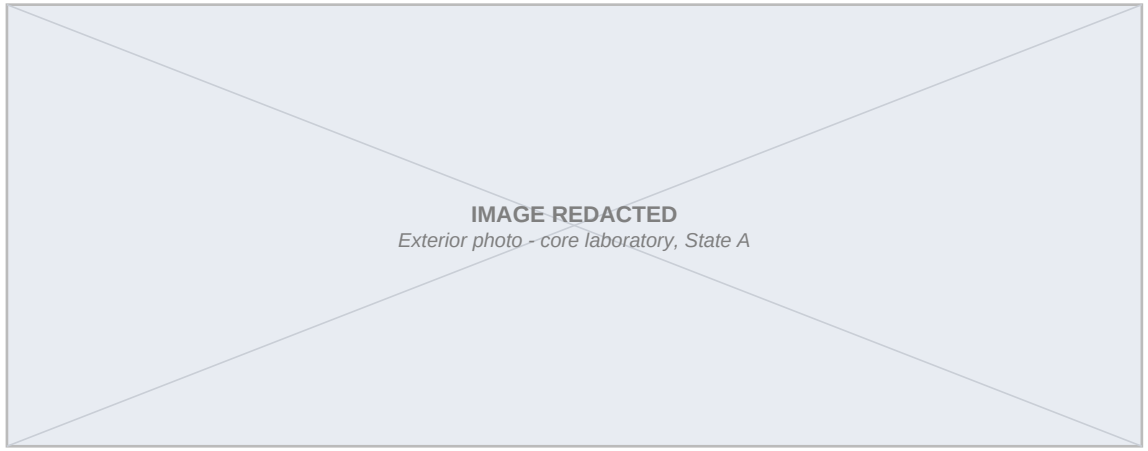
The Company has grown without external capital, funding every expansion of the test menu, courier network and instrument base from operating cash flow. Revenue has compounded at 8.5% annually since 2024A while adjusted EBITDA grew 13.1% in 2025A and is estimated to grow a further 14.5% in 2026E, reflecting fixed-cost absorption as accession volume rose from 85,204 to an estimated 101,458. Management has already recruited a Laboratory Manager (December 2026) and budgeted a shift supervisor, an IT lead, two couriers and additional accessioners for 2027 to support the next phase.



Source: Company management.

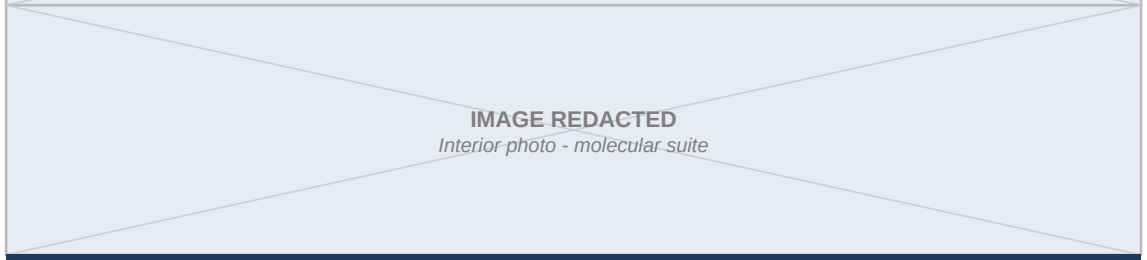
Facility Overview

A centralised, accredited 24,000 sq. ft. laboratory engineered for 520 accessions per single shift



- 24,000 sq. ft. leased core laboratory housing 62 analytical instruments across chemistry, haematology, immunoassay, toxicology (LC-MS/MS) and molecular (real-time PCR) benches, with a dedicated accessioning floor and specimen archive.
- Staffed by 41 laboratory professionals across three departments; the facility runs a single extended shift, processing 379 accessions per working day in 2026E against a validated single-shift ceiling of 520.
- The lease includes a right of first refusal on 9,500 sq. ft. of adjacent space and a pre-approved second-floor build-out, allowing capacity to be doubled without relocating instruments or re-validating assays.



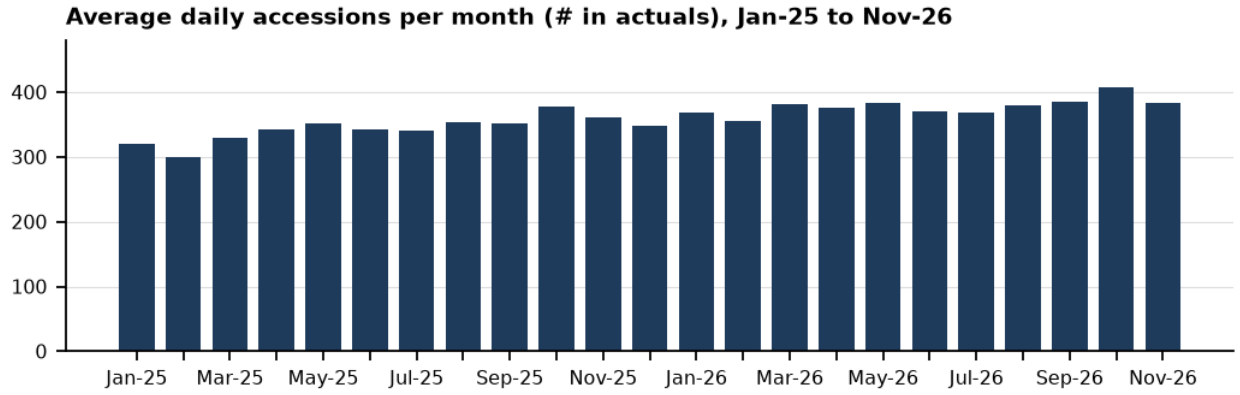


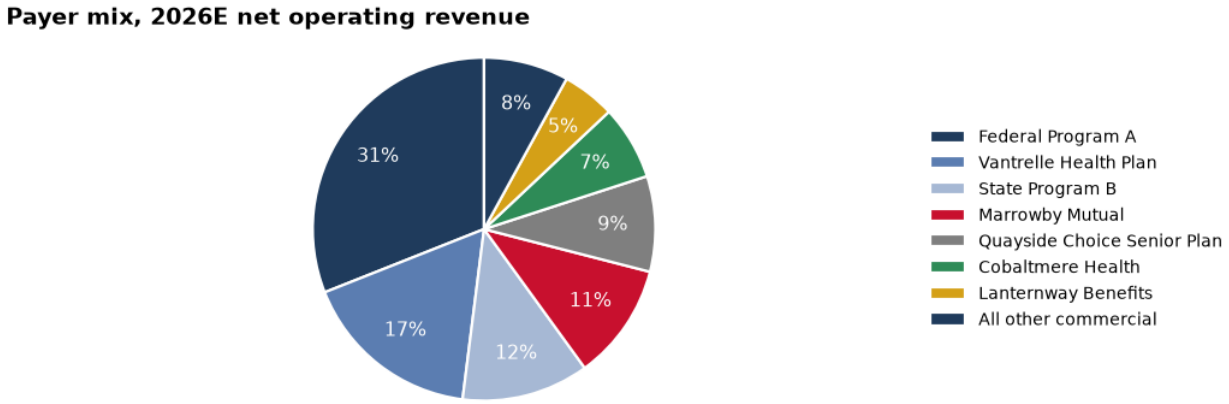
Facility metric	Value
Core laboratory area	24,000 sq. ft.
Analytical instruments	62
Validated single-shift capacity	520 accessions / day
2026E average daily accessions	379 (73% utilisation)
Patient service centres	6
Courier vehicles / routes	11 / 14

Source: Company management.

Diversified Customer & Payer Mix

In-network relationships with government programmes and seven commercial payers limit concentration risk





Contracted payers

Federal Program A	**Vantrelle Health Plan**	**State Program B**
Marrowby Mutual	**Quayside Choice Senior Plan**	**Cobaltmere Health**
Lanternway Benefits		

Volume momentum

Average daily accessions rose from 320 in January 2025 to 384 in November 2026, with October 2026 the first month above 400. The increase has been absorbed within the existing shift structure and courier network; 2026E utilisation stands at 73% of the 520-per-day single-shift ceiling.

Balanced reimbursement

Government programmes account for 43% of 2026E net revenue and no single commercial payer exceeds 17%. All seven commercial contracts are in-network, and the net-to-gross collection rate has held between 42.0% and 42.5% across 2024A-2026E, a mix that mirrors the regional payer demographics of the Company's client base.

(1) Payer mix based on 2026E net operating revenue. (2) Collection rate = net operating revenue / gross charges. Source: Company management; QoE report (QoE Advisor) through Aug-26; Falkmere analysis.

Well-Established Management and Operational Leadership

A hands-on leadership bench and institutional infrastructure that can support a materially larger platform



Dr. Samuel Achterberg

Founder & Chief Executive Officer

Clinical chemist (PhD) who founded the Company in 2009 after twelve years directing hospital laboratories. Sets test-menu strategy, leads payer negotiations and chairs the quality committee. Retains 70% of the membership interests.



Dr. Miriam Oyelaran

Laboratory Director

Board-certified clinical laboratory director (PhD) with the Company since 2016. Responsible for assay validation, proficiency testing, the 41-person technical bench and the accreditation programme; led the 2019-20 molecular build-out.



Janet Kowalczyk

Director of Operations

With the Company since 2012, responsible for the 6 patient service centres, the courier fleet, accessioning and client services. Designed the routing model that supports same-day pickups across the current footprint.



Karen Liu-Bennett

Chief Financial Officer

CPA who joined in 2018 from a regional healthcare services group where she was controller. Owns budgeting, revenue-cycle oversight, the outsourced billing relationship and the QoE process; introduced monthly management reporting in 2019.



Victor Ramos

Vice President, Sales

Joined in 2014 and built the eight-person sales organisation. Manages the physician-practice, urgent-care and behavioural-health channels and personally originated the Silvermoss Senior Living relationship; leads the March 2027 representative hires.



Dr. Anil Devarakonda

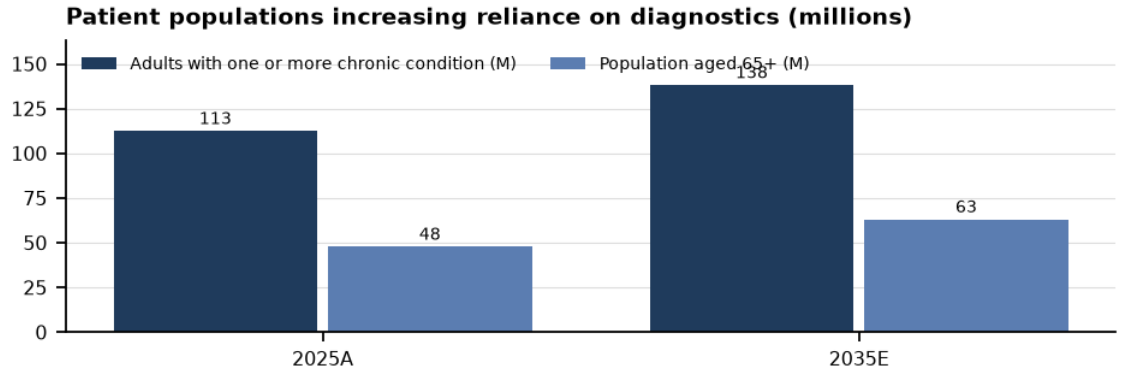
Medical Director (transitioning)

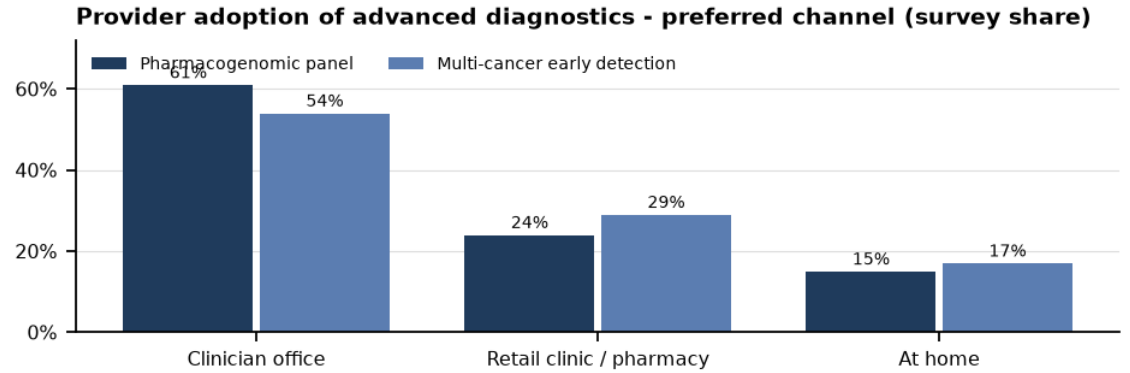
Board-certified pathologist and Medical Director since 2011, providing clinical oversight and consultative pathology. Will work through December 2026 before retiring; a successor search is under way and the budget reflects the transition from January 2027.

Source: Company management.

Highly Attractive Market Tailwinds

A high-growth diagnostics ecosystem fuelled by demographics, chronic disease management and preventive care



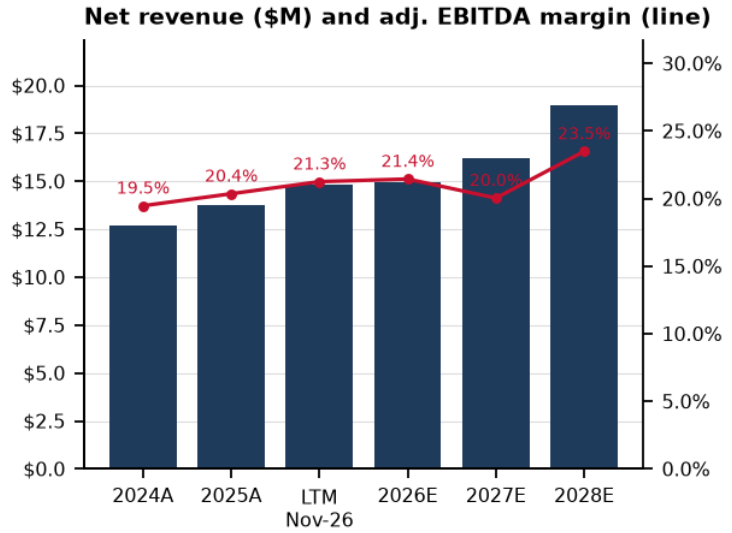


- Adults living with at least one chronic condition are projected to grow at a 1.7% CAGR to 2035; each such patient generates on average 3.4 laboratory orders per year, creating a recurring, non-discretionary testing base.
- The 65+ cohort grows at 2.6% annually and consumes roughly twice the tests per capita of the working-age population, the demographic that underpins the Company's senior-living channel.
- Consolidation wave favours scaled independent labs
  - Roughly 2,750 independent laboratories operate nationally; 131 changed hands in 2024-25 as national chains rationalised outreach networks.
  - Regional operators with automation and same-day logistics win share when hospital outreach programmes are divested or closed.
  - The Company's turnaround advantage positions it as a natural acquirer of sub-scale local competitors.
- Consumer demand for novel diagnostics
  - 61% of surveyed adults would take a pharmacogenomic panel if ordered by their clinician, a channel the Company already serves through its molecular department.
- Shift to preventive and value-based care
  - Risk-bearing physician groups need low-cost, fast-turnaround testing to close care gaps; two IPAs representing 640 physicians are in the Company's active pipeline.
- Hospital outsourcing of outreach laboratories
  - Health systems increasingly exit outreach testing; the Company's 12-24 hrs turnaround is well inside the 24-48 hour benchmark of national reference laboratories.

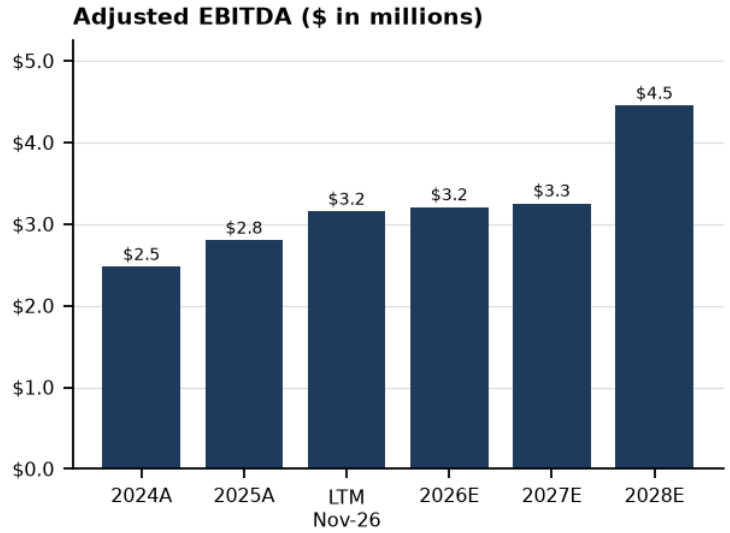
Sources (synthetic): (1) Industry Research Firm A. (2) National statistics agency population projections. (3) Consulting Firm B laboratory M&A tracker. (4) National laboratory chain annual report. (5) Regulator statistics.

Strong Upward Financial and Operational Trends

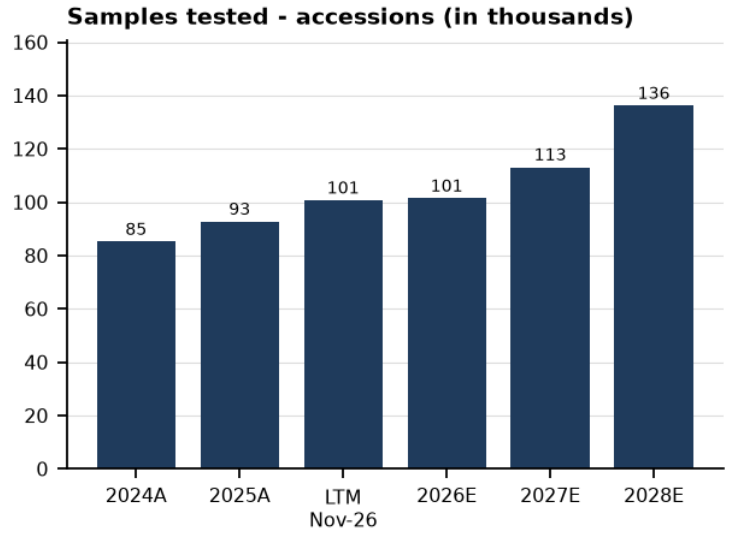
Revenue CAGR of 10.5% and adjusted EBITDA CAGR of 15.8% from 2024A to 2028E on rising volume and stable reimbursement



- Revenue grew 8.2% in 2025A and 8.7% in 2026E; the 2028E step-up reflects the full-year contribution of representatives hired in March 2027.
- Margins expand from 19.5% to 23.5% as volume is absorbed by a largely fixed cost base.



- Adjusted EBITDA of $3.2M in 2026E, up 14.5%; 2027E is held near flat as the Company invests $136K in new representatives and $448K in supervisory, IT, courier and accessioning hires ahead of volume.
- 2028E adjusted EBITDA of $4.5M at a 23.5% margin.



- Accessions rise from 85,204 in 2024A to 136,217 in 2028E, a 12.4% CAGR.
- Implied net reimbursement per accession eases from $149.19 to $139.08 as senior-living and new-account volume, priced at $102.00-$112.00, becomes a larger share of the mix.

(1) 2024A-25A and Jan-Aug 2026 per QoE report; Sep-Nov 2026 management-reported; Dec-26 onward projections. LTM Nov-26 not seasonally adjusted. Source: Company management; QoE report (QoE Advisor) through Aug-26; Falkmere analysis.

Actionable Growth Levers

A multi-pronged growth strategy that builds on the existing sales force, new representatives, institutional channels and geography

Deepen penetration through the existing sales force

Eight representatives manage 338 active accounts. Cross-selling molecular and toxicology panels into routine-only practices lifts revenue per account without new logistics cost; the existing team is modelled to hold $13.7M of 2026E revenue essentially flat through 2028E ($13.9M), so any upsell is upside to plan.

Accelerate growth via new sales representatives and new testing areas

Two experienced representatives start in March 2027 as non-compete periods with consolidated competitors expire. At 55 accessions per working day in 2027E rising to 119 in 2028E and $112.00 per accession, they contribute $1.4M in 2027E and $3.6M in 2028E for a fully loaded cost of $136K and $356K.

Broaden access via IPA and institutional partnerships

The Silvermoss Senior Living contract (42 communities) went live in April 2025 and is projected at $1.4M in 2027E and $1.5M in 2028E. Two independent physician associations with a combined 640 physicians are in late-stage discussions, offering the same recurring, route-dense volume profile.

Enter high-potential geographic markets

The draw-station model, existing multi-state licensure and overnight courier partners allow low-capex entry into adjacent Northeast metros and, longer term, into high-growth southern states with favourable commercial reimbursement and rapidly growing senior populations. None of this expansion is in the projections.

Source: Company management.

Key Investment Considerations

Six reasons the Company is a compelling platform for a strategic or financial acquirer

1. Scalable platform with capacity headroom

73% single-shift utilisation in 2026E, a pre-approved second-floor build-out and a validated ceiling of 520 daily accessions before any second shift.

2. Rapid, visible growth from a commercial engine

Revenue projected to grow from $15.0M in 2026E to $18.9M in 2028E on contracted senior-living volume and two funded representative hires.

3. Strong reimbursement visibility and a diversified test mix

Government programmes 43%, seven in-network commercial payers and four service lines; collection rate stable at 42.5% of gross charges.

4. Demonstrated operational excellence

12-24 hrs routine turnaround, 99.6% first-pass accuracy and clean accreditation history since 2016.

5. Technology-forward infrastructure

LIS/EMR interfaces with 210 client systems, automated chemistry and molecular lines, and IT spend of $238K in 2026E supporting structural margin advantages.

6. Experienced, hands-on management bench

Six leaders averaging 13 years of tenure who have delivered every stage of growth from operating cash flow, with a planned Medical Director transition and a Laboratory Manager already hired.

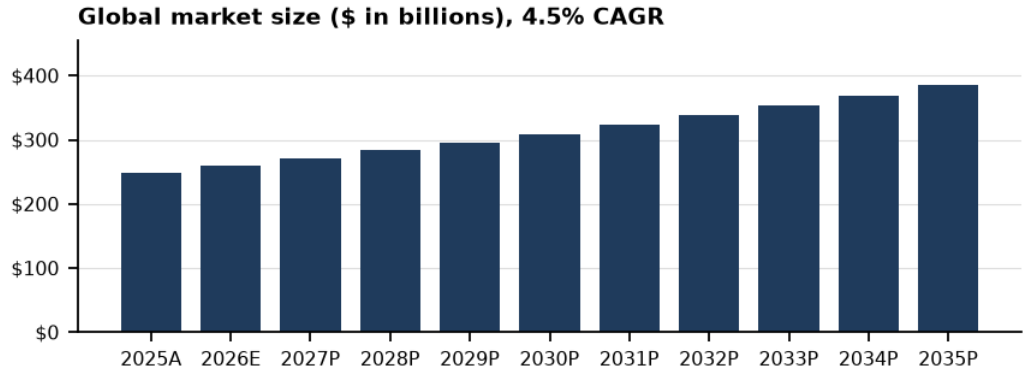
Source: Company management.

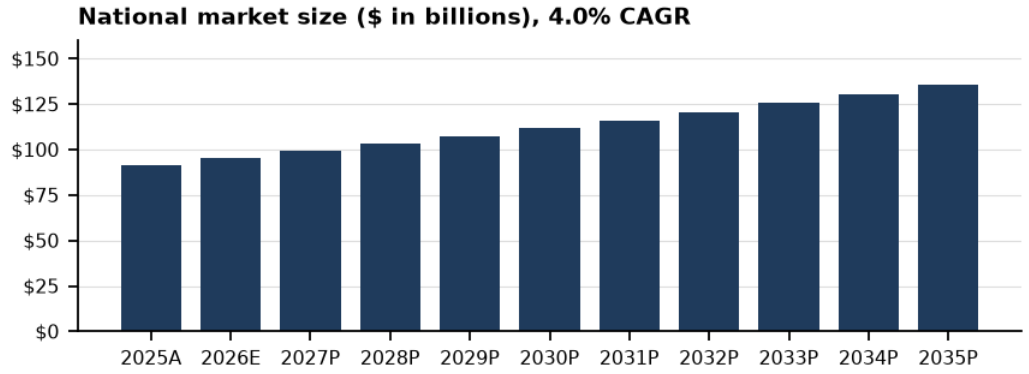
Section 2

Industry Overview

Industry Overview

Secular demand growth, technology-driven complexity and consolidation favour accredited regional laboratories





The clinical laboratory testing market comprises routine chemistry, haematology and immunoassay testing, molecular and genetic diagnostics, anatomic pathology and toxicology performed by hospital laboratories, national reference chains and independent regional laboratories. National spend of $91.7 billion in 2025 is projected to reach $135.7 billion by 2035, with independent regional labs the fastest-growing setting.

- Demand expansion
  - Chronic-disease monitoring and preventive screening protocols add recurring orders per patient.
  - Employer and value-based programmes are widening the tested population.
- Technology and complexity
  - Molecular, pharmacogenomic and mass-spectrometry assays are migrating from academic centres to regional labs.
  - Complexity favours accredited operators with validated menus.
- Operational structure
  - Hub-and-spoke logistics with draw sites and couriers is the winning model for physician-office volume.
  - Same-day turnaround is a differentiator against national chains.
- Reimbursement and automation
  - Fee-schedule pressure is offset by automation and per-test cost reduction.
  - In-network status with commercial payers is increasingly decisive.
- M&A drivers
  - National chains and sponsors are consolidating sub-scale independents.
  - Hospitals are divesting outreach programmes to specialists.
- Digitisation and AI integration
  - LIS/EMR connectivity and AI-assisted QC lower error rates and labour intensity.
  - Data-rich labs can offer population-health analytics to IPAs.

Sources (synthetic): (1) Industry Research Firm A. (2) Industry Research Firm B. (3) Consulting Firm B. (4) Trade association statistics. (5) Regulator fee-schedule data. (6) Company management.

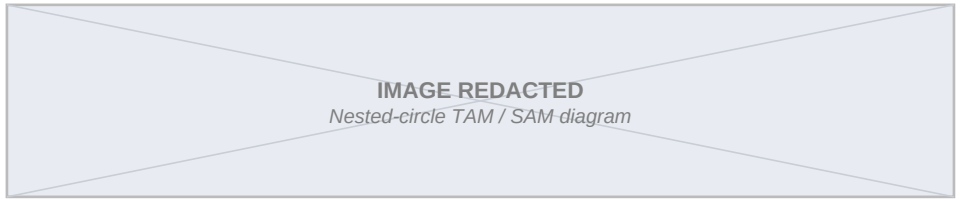
Market Opportunity & Reach

A fragmented, high-growth market in which the Company holds a fraction of one percent of its serviceable footprint

$91.7B
TAM - national clinical laboratory testing spend, 2025A

$16.1B
SAM - population-weighted share (17.6%) of the current licensed Northeast footprint

0.09%
2026E net revenue as a share of SAM



Market size

The national market of $91.7 billion forms the TAM. Weighting that figure by the population of the seven states in which the Company is licensed yields a SAM of $16.1 billion. Against 2026E net revenue of $15.0M the Company holds 0.09% of its SAM, so each additional 0.1 percentage point of share represents roughly $16 million of revenue.

TAM and SAM rationale

TAM is defined as all national clinical laboratory testing spend and is projected to grow at 4.0% annually to $135.7 billion by 2035. SAM is calculated as the TAM multiplied by the share of the national population resident in the states where the Company holds licensure and courier coverage. Obtainable share rises with each new draw site, IPA contract and payer credential, and the SAM itself expands as licences are added in target southern growth states.

- Independent regional laboratories hold about 26% of national volume, split among roughly 2,750 operators, few of which combine accreditation, molecular capability and a courier network.
- The Company's pipeline (two IPAs, 640 physicians; three senior-living operators) would add an estimated 0.05 percentage points of SAM share on its own.

(1) Industry Research Firm A (synthetic); population weights per national statistics agency.

Section 3

Service Offerings

Delivery Model

A high-density array of routine, molecular, pathology and toxicology services delivered from one accredited site

71% Routine chemistry & haematology	**19%** Molecular / PCR
6% Anatomic pathology send-outs	**4%** Toxicology

- 410 unique tests across chemistry, haematology, coagulation, immunoassay, urinalysis, toxicology (LC-MS/MS confirmation), molecular/PCR (respiratory, women's health, pharmacogenomics) and anatomic pathology send-outs; 14 custom panels for behavioural-health, geriatric and urgent-care clients.
- In-network with Federal Program A, Vantrelle Health Plan, State Program B, Marrowby Mutual, Quayside Choice Senior Plan, Cobaltmere Health, Lanternway Benefits; credentialed with Federal Program A and State Program B since 2010.
- Consultative services from board-certified laboratory and medical directors, including interpretive toxicology reports and antibiotic stewardship summaries for senior-living clients.
- 99.6% first-pass reporting accuracy and 12-24 hrs turnaround on routine work versus a 24-48 hour benchmark for national reference laboratories.

Department	Representative tests	Platform	Routine TAT
Chemistry	Metabolic panels, lipids, HbA1c, thyroid, vitamin D	Automated chemistry / immunoassay line	Same day
Haematology & coagulation	CBC with differential, PT/INR, ESR	Five-part differential analysers	Same day
Toxicology	Drug screens with LC-MS/MS confirmation, medication monitoring	Immunoassay screen + LC-MS/MS	24 hrs
Molecular / PCR	Respiratory panels, STI, UTI, pharmacogenomics	Real-time PCR, two suites	24 hrs
Microbiology	Cultures, sensitivities, C. difficile	Automated ID/AST	48-72 hrs
Anatomic pathology	Biopsies, Pap/HPV co-testing (send-out with in-house accessioning)	Partner pathology group	3-5 days

Mission

To deliver accurate, timely and clinically useful results that let community physicians and care operators make better decisions for their patients, every day, at a cost the healthcare system can sustain.

Source: Company management.

Strategic Positioning & Competitive Advantages

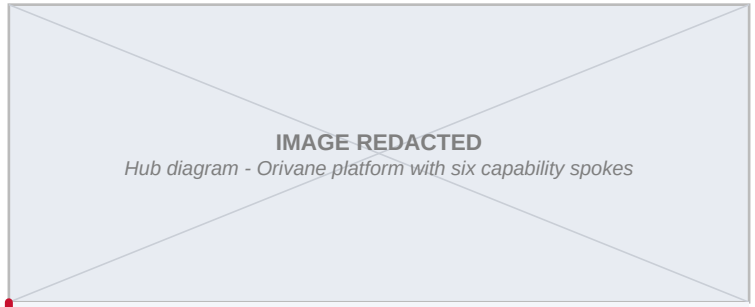
Six reinforcing capabilities position the Company for accelerated expansion under new ownership

Operator-driven platform

Founder-led since 2009 with a leadership team that runs the bench, the routes and the billing process personally; decisions on test menu, pricing and client service are made in days.

Comprehensive high-complexity testing expertise

410 validated tests including LC-MS/MS toxicology and real-time PCR panels normally found only in national reference laboratories, allowing clients to consolidate vendors.



Deep operational and scientific leadership

Board-certified laboratory and medical directors, a 41-person technical bench and a Director of Operations who designed the courier routing model.

Quality-driven, compliance-focused operations

Independent accreditation-body accreditation without deficiency since 2016, documented proficiency-testing performance and a compliance programme audited annually by external counsel.

Broad regional footprint and integrated logistics

6 patient service centres, 11 courier vehicles on 14 routes and overnight partners deliver same-day pickup across the licensed Northeast footprint.

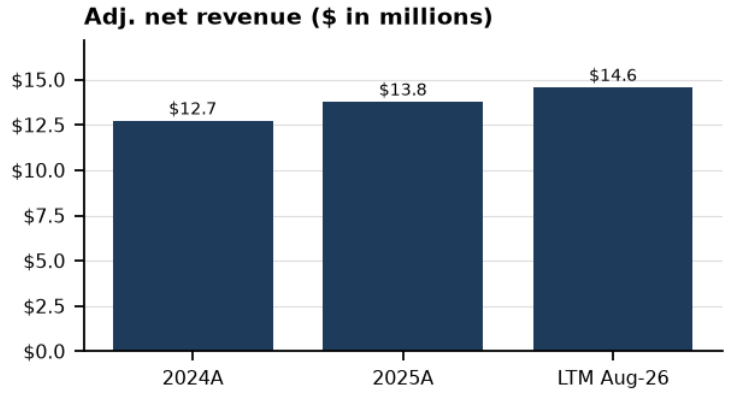
Scalable infrastructure and expansion readiness

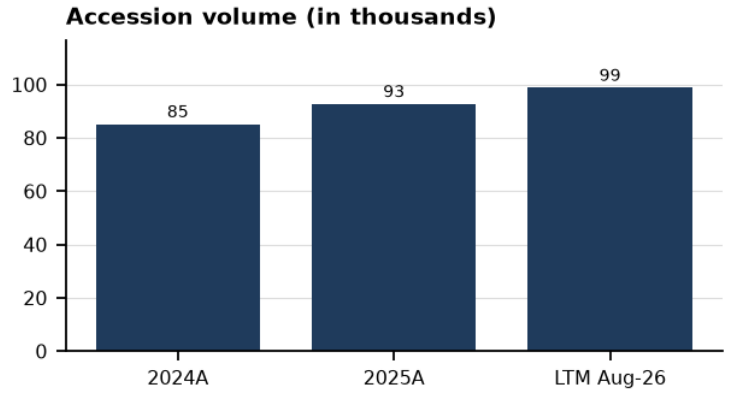
73% utilisation of a 520-per-day single shift, a pre-approved second-floor build-out and an LIS already interfaced with 210 client systems.

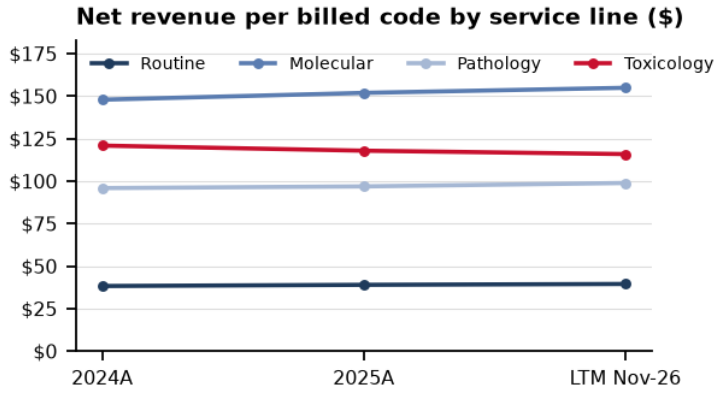
Source: Company management.

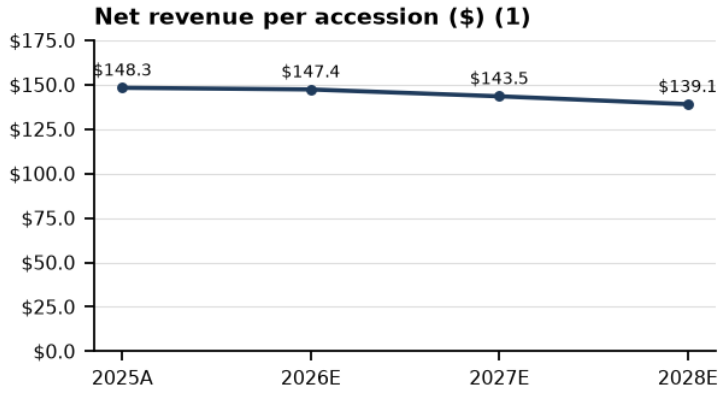
Reimbursement and KPIs

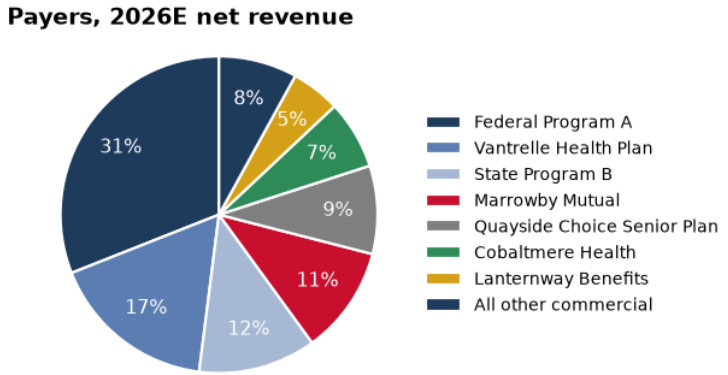
Predictable, in-network reimbursement with stable net revenue per accession and a rising collection rate











- Collection rate of 42.2%, 42.0% and 42.5% of gross charges in 2024A, 2025A and 2026E; gross charges of $35.2M in 2026E.
- Bad-debt expense of $131K in 2026E, 0.9% of net revenue.
- Net revenue per accession declines modestly to $139.08 by 2028E purely from mix as lower-priced senior-living and new-account volume grows; no fee-schedule cuts are assumed.

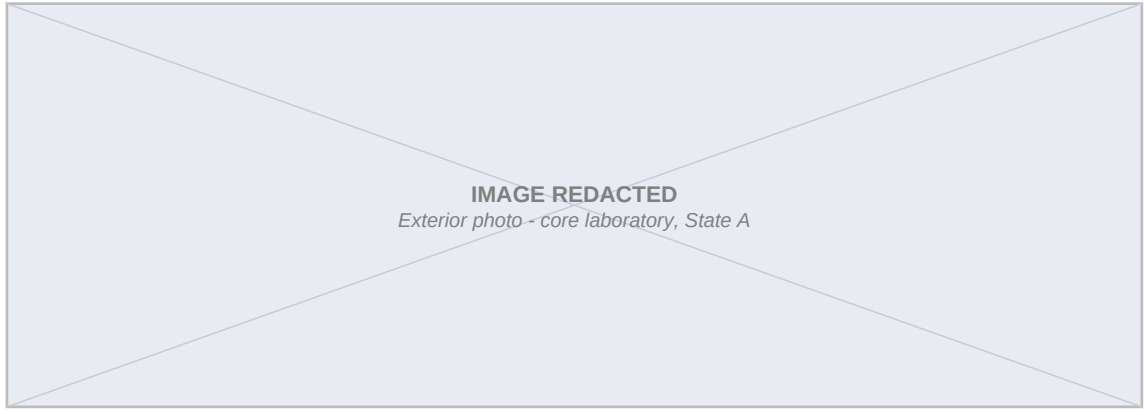
(1) Per-accession figures assume the 2026E test mix within each channel; per-procedure-code rates are management estimates. Source: Company management; QoE report (QoE Advisor) through Aug-26; Falkmere analysis.

Locations Overview

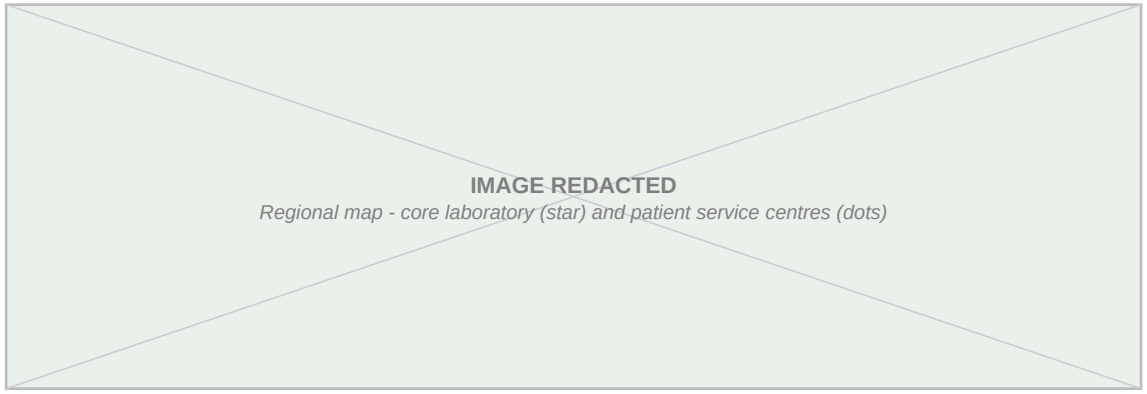
One centralised laboratory supported by 6 regional patient service centres and a company-owned courier network

Ample capacity for increased testing volume

The core laboratory handled 379 accessions per working day in 2026E, 73% of the validated 520 single-shift ceiling. Management's plan takes utilisation to 98% by 2028E, at which point the pre-approved second-floor build-out and a second accessioning shift would lift capacity to approximately 1,040 accessions per day.



24,000 sq. ft. core laboratory and flagship patient service centre - Headquarters, State A



Core laboratory - State A; PSC 1-4 - State A metro; PSC 5 - State B; PSC 6 - State C

- Facility 1 - Headquarters, State A
  - Core laboratory, accessioning, courier hub
  - Flagship patient service centre (extended hours)
- Facilities 2-5 - State A metropolitan area
  - Four leased patient service centres of 900-1,400 sq. ft.
  - Two-hour courier loops to the core laboratory
- Facility 6 - State B
  - Patient service centre co-located with an IPA client
  - Overnight courier partner
- Facility 7 - State C
  - Patient service centre opened 2022 to serve behavioural-health clients

Source: Company management.

Proven Results Lead to Lasting Relationships

Dependable diagnostics build trust: net logo retention of 103% over the trailing twelve months (1)

Practice administrator, multi-site primary-care group

"We moved our full routine and molecular volume to Orivane in 2023. The laboratory director calls us directly on critical values, and our clinicians trust the results without re-testing."

Medical director, urgent-care network

"Results are in our EMR before the next morning's huddle. Turnaround and the responsiveness of the client-services desk are why we have not looked at anyone else."

Director of clinical services, Silvermoss Senior Living

"The interpretive toxicology and antibiotic stewardship reports changed how our nurses manage residents. Orivane behaves like part of our clinical team."

Office manager, behavioural-health practice

"Couriers arrive when they say they will, every day, and the custom panel Orivane built for us cut our send-outs to two vendors from five."

Physician, independent practice association

"I recommended Orivane to the other practices in our IPA for pharmacogenomic testing. The pricing was fair and the reports are readable by a busy clinician."

Client relationships are long-lived: the ten largest accounts have averaged nine years with the Company, and 338 unique clients ordered in the year to date, up from 286 in 2024A.

(1) Net logo retention measures trailing-twelve-month revenue from clients active twelve months earlier, counting clients above $5,000 of annual billings. Quotes are illustrative and anonymised. Source: Company management.

Sample Testing & Processing Overview

A standardised, LIS-tracked, end-to-end process from specimen collection to electronic result delivery

Step 1 - Collect	Step 2 - Transport & accession	Step 3 - Analyse	Step 4 - Report
Specimen drawn at client site or PSC	Courier pickup, barcode accessioning	Tests run per order on validated platforms	Results released to EMR / portal
1. Sample collected	**2. Sample shipped and received**	**3. Tests run per order**	**4. Results reported and disseminated**
Phlebotomists at the six PSCs or client staff collect the specimen against an electronic order placed through the LIS portal or an EMR interface. Requisition data, payer eligibility and medical-necessity checks are validated at the point of collection, reducing downstream billing rejections.	Company couriers run 14 fixed routes with two-hour loops in the metro area; State B and State C specimens travel overnight with a partner. On arrival, each tube is barcoded and accessioned into the LIS within 20 minutes, with temperature and chain-of-custody logged automatically.	Orders route to the chemistry, haematology, toxicology, molecular or microbiology benches. 62 instruments are interfaced bidirectionally to the LIS; auto-verification releases in-range routine results, while directors review flagged values and all confirmatory toxicology.	Results post electronically to 210 interfaced client systems or the secure portal, with critical values phoned within 30 minutes. 99.6% of reports are released without amendment; routine work is reported in 12-24 hrs, and claims are generated the same night through the outsourced RCM platform.

12-24 hrs	< 20 min	210	4.1%
Routine turnaround	Receipt to accessioning	Interfaced client EMR / practice systems	Claims requiring re-work (2026E)

- Information technology: a cloud-hosted LIS (upgraded 2022) with bidirectional instrument interfaces, a clinical-messaging integration engine and a client portal; 2026E IT expense of $238K and an IT lead budgeted from March 2027 to support AI-assisted quality control and automated accessioning.

System	Purpose	Deployment
Laboratory information system (LIS)	Order entry, accessioning, instrument interfaces, auto-verification, result release	Cloud-hosted, upgraded 2022
Clinical-messaging integration engine	Bidirectional interfaces to 210 client EMR and practice-management systems	Cloud-hosted
Client portal	Web ordering, result retrieval, supply requests for non-interfaced clients	Cloud-hosted
Revenue-cycle platform	Claim scrubbing, submission, remittance posting and denial management	Outsourced RCM vendor
Courier routing and chain-of-custody	Route optimisation, pickup confirmation, temperature logging	Mobile application

Source: Company management.

Section 4

Organizational & Management Overview

Organizational Overview

An organisation of 95 employees built on distinct laboratory, commercial, logistics and finance disciplines

Dr. Samuel Achterberg - Founder & CEO

Karen Liu-Bennett - CFO	Dr. Miriam Oyelaran - Laboratory Director	Dr. Anil Devarakonda - Medical Director (transitioning)	Victor Ramos - VP Sales	Janet Kowalczyk - Director of Operations

Department / function	Reports to	Lead
Chemistry, haematology & coagulation	Laboratory Director	Core Laboratory Manager (hired Dec-26)
Toxicology (LC-MS/MS)	Laboratory Director	Toxicology Supervisor
Molecular / PCR	Laboratory Director	Molecular Supervisor
Microbiology & pathology send-outs	Medical Director	Microbiology Lead
Quality assurance / quality control	Laboratory Director	QA/QC Manager
Accessioning & client services	Director of Operations	Processing / Shift Supervisor (Feb-27)
Phlebotomy & patient service centres	Director of Operations	PSC Coordinator
Logistics & couriers	Director of Operations	Logistics Coordinator
Billing, RCM oversight & finance	CFO	Billing Manager
Information technology	CFO	IT Lead (Mar-27)

Headcount by function	#
Executives & senior management	6
Laboratory professionals (technologists, technicians, pathology support)	41
Phlebotomy & patient service centre staff	18
Logistics & couriers	9
Billing, RCM & customer service	7
Sales representatives	8
Information technology	3
Finance & administration	3
Total	**95**

- Budgeted 2027 additions: Processing/Shift Supervisor ($110K, Feb-27), two couriers ($50K each, Feb-27), IT Lead ($180K, Mar-27), Inventory Coordinator ($75K, Mar-27) and additional accessioners ($43K in 2027E, $152K in 2028E).
- Laboratory Manager ($200K) started in December 2026; the Medical Director role ($150K) transitions out from January 2027 with clinical oversight covered by the Laboratory Director and contracted pathologists during the successor search.
- Total salaries and wages of $3.7M in 2026E rise to $4.3M in 2027E and $4.6M in 2028E, 24.3% of revenue.

Source: Company management.

Management

An experienced leadership team with deep tenure and clear accountability across science, operations, sales and finance



Dr. Samuel Achterberg

Founder & Chief Executive Officer

Clinical chemist (PhD) who founded the Company in 2009 after twelve years directing hospital laboratories. Sets test-menu strategy, leads payer negotiations and chairs the quality committee. Retains 70% of the membership interests. Holds a PhD in clinical chemistry and has served on the state clinical laboratory advisory board since 2019. Intends to remain with the business through a transition period and is open to a meaningful rollover.



Karen Liu-Bennett

Chief Financial Officer

CPA who joined in 2018 from a regional healthcare services group where she was controller. Owns budgeting, revenue-cycle oversight, the outsourced billing relationship and the QoE process; introduced monthly management reporting in 2019. Previously spent six years in public accounting auditing healthcare providers. Manages the Company's banking relationship and the accounts-payable and payroll functions.



Dr. Miriam Oyelaran

Laboratory Director

Board-certified clinical laboratory director (PhD) with the Company since 2016. Responsible for assay validation, proficiency testing, the 41-person technical bench and the accreditation programme; led the 2019-20 molecular build-out. Holds national certification as a high-complexity laboratory director and oversees the annual proficiency-testing programme covering all analytes.



Victor Ramos

Vice President, Sales

Joined in 2014 and built the eight-person sales organisation. Manages the physician-practice, urgent-care and behavioural-health channels and personally originated the Silvermoss Senior Living relationship; leads the March 2027 representative hires. Prior to joining, spent nine years in diagnostics sales with a national reference laboratory. Compensation is salary plus commission tied to net collected revenue.



Janet Kowalczyk

Director of Operations

With the Company since 2012, responsible for the 6 patient service centres, the courier fleet, accessioning and client services. Designed the routing model that supports same-day pickups across the current footprint. Previously managed a regional medical courier business. Responsible for the PSC lease portfolio, fleet and the accessioning hires planned for 2027.



Dr. Anil Devarakonda

Medical Director (transitioning)

Board-certified pathologist and Medical Director since 2011, providing clinical oversight and consultative pathology. Will work through December 2026 before retiring; a successor search is under way and the budget reflects the transition from January 2027. Fellowship-trained in clinical pathology; will remain available on a consulting basis to support the transition to a successor Medical Director.

Source: Company management.

Section 5

Financial Overview & Growth Opportunities

Growth Opportunities Overview

A targeted, funded strategy to extend the Company's reach across sales pipeline, laboratory capacity and geography

Expansion and execution of the sales pipeline

Existing representatives, two funded hires from March 2027 and the institutional senior-living channel.

- Existing eight representatives are modelled flat at $13.7M-$13.9M of annual revenue; organic upsell is upside.
- Two new representatives contribute $1.4M in 2027E and $3.6M in 2028E as non-competes with consolidated competitors expire.
- Silvermoss Senior Living grows from $1.2M in 2026E to $1.5M in 2028E; two IPAs with 640 physicians in late-stage discussion.
- Mix shift toward higher-value molecular and toxicology panels in every channel.

Laboratory facility expansion

Incremental build-out within the existing lease to double daily capacity ahead of 2028E utilisation.

- 2026E utilisation of 73% rises to 83% in 2027E and 98% in 2028E on the current 520-per-day ceiling.
- Pre-approved second-floor build-out (drawings complete) and a right of first refusal on 9,500 sq. ft. of adjacent space.
- Layout designed for a second molecular suite and expanded LC-MS/MS bench; automated accessioning and AI-assisted QC preserve margin as volume doubles.
- A Laboratory Manager (Dec-26), Shift Supervisor (Feb-27) and IT Lead (Mar-27) are already budgeted.

Geographic expansion

Draw-station-led entry into adjacent Northeast metros and, longer term, high-growth southern states.

- Existing licensure in seven states and credentialing with the major national payers reduce time-to-revenue in new markets.
- A patient service centre can be opened for under $60K of capex and staffed with two phlebotomists, keeping expansion asset-light.
- Target states combine favourable commercial reimbursement with fast-growing senior populations suited to molecular and toxicology testing.
- Geographic expansion is not included in the 2027E-2028E projections.

Source: Company management.

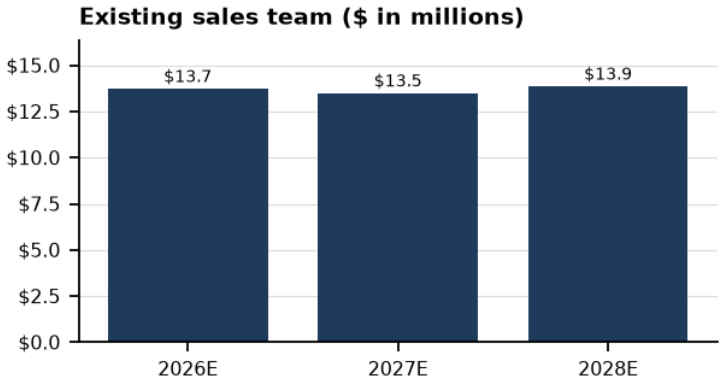
Growth Opportunities - Pipeline Execution & Expansion

Three complementary growth engines take net revenue from $15.0M in 2026E to $18.9M in 2028E

Existing sales team

Eight representatives; 334 accessions per working day in 2026E

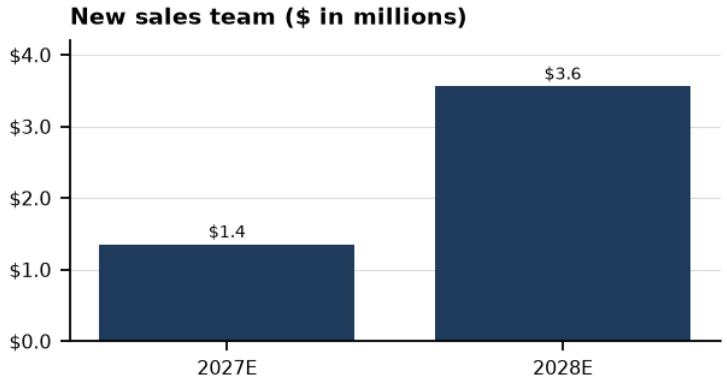
- Baseline production
  - 89,386 accessions at $153.51 per accession in 2026E, $13.7M of revenue.
  - Modelled at 333-336 accessions per day through 2028E with pricing held at $154.43.
- Operating leverage
  - Sales representative expense for the current team of $747K in 2026E grows only with inflation.



New sales team

Two representatives starting March 2027

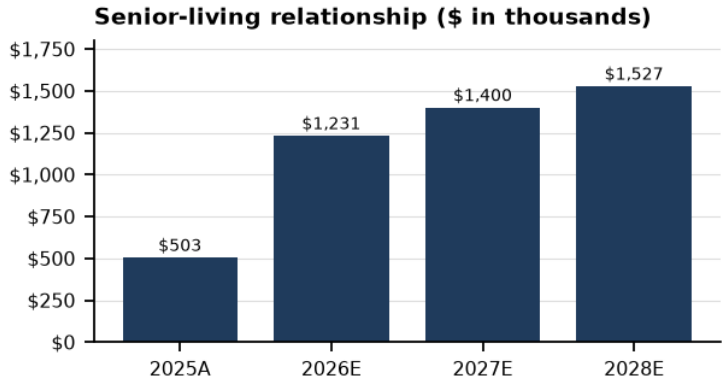
- Immediate uplift
  - 55 accessions per working day over 221 working days in 2027E (12,110 accessions), rising to 119 per day in 2028E (31,820).
  - Priced at $112.00 per accession, reflecting a routine-heavy initial book.
- Cost and visibility
  - Fully loaded cost of $136K in 2027E and $356K in 2028E, including commissions.
  - Both candidates identified; start dates set by non-compete expiry.



Institutional (senior-living) relationships

Silvermoss Senior Living - 42 communities, live since April 2025

- Recurring volume
  - 12,072 accessions in 2026E at $102.00 per accession ($1.2M), growing to 14,685 accessions and $1.5M in 2028E.
- Operational fit and ramp
  - Scheduled route-dense draws; ramp from 19 to 55 accessions per working day is contracted, not assumed.
  - Three further senior-living operators in discussion.

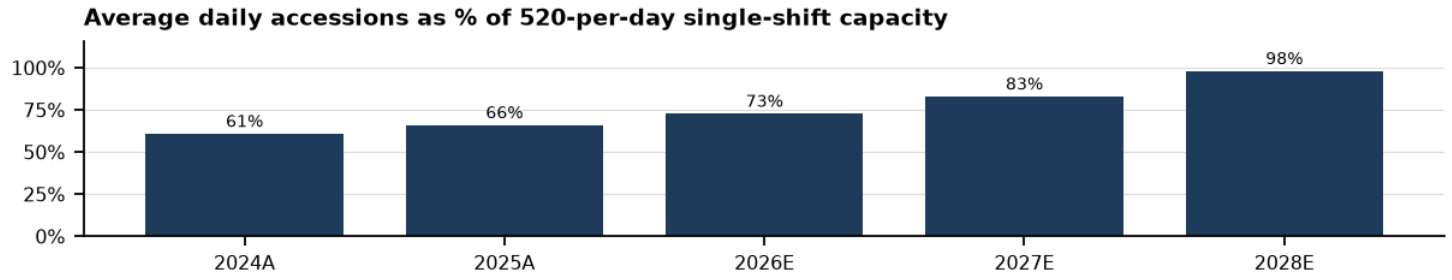


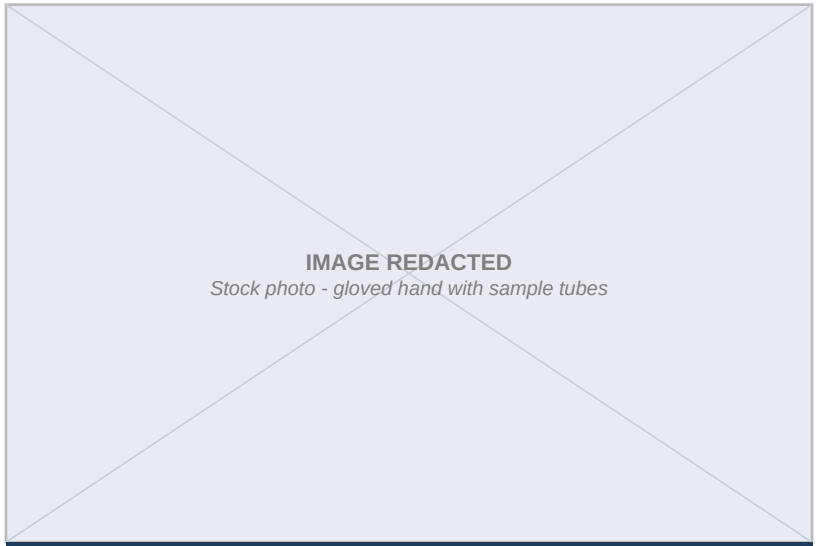
Source: Company management projections (Revenue Build).

Growth Opportunities - Buildout & Expansion of Capacity

Investing in infrastructure and automation ahead of demand to double daily capacity within the existing footprint

- Operational scale-up opportunity
  - Current throughput of 379 accessions per working day (2026E) against a validated single-shift ceiling of 520; a full second shift would take the same floor to roughly 1,040.
  - Utilisation of 61% in 2024A, 73% in 2026E and 98% in 2028E under the management plan.
  - AI-assisted QC and automated accessioning (IT Lead from March 2027) reduce manual touchpoints per specimen by an estimated 30%.
  - Revenue capacity of the current shift at 2026E rates is approximately $20.5M (1), against 2026E revenue of $15.0M.
- Post-expansion state
  - Right of first refusal on 9,500 sq. ft. of adjacent tenant space within the same building.
  - Second-floor build-out of 6,200 sq. ft. with drawings complete and landlord consent obtained; estimated capex of $1.4M phased over 2027-28.
  - All added space dedicated to laboratory operations: second molecular suite, expanded toxicology bench and a dedicated accessioning floor.
  - Total footprint of approximately 39,700 sq. ft. with daily capacity doubled to about 1,040 accessions.





Capacity metric	2026E	2028E
Working days	268	267
Average daily accessions	379	510
Single-shift ceiling	520	520
Utilisation	72.8%	98.1%
Net revenue per accession	$147.38	$139.08

(1) Revenue capacity = 520 daily accessions x $147.38 net revenue per accession x 268 working days. Source: Company management projections (Revenue Build).

Growth Opportunities - Geographic Expansion

Expanding the footprint through draw sites, payer credentialing and logistics rather than new laboratories

Current footprint - home-state platform

Core laboratory and 6 patient service centres cover State A and adjacent metros in States B and C with same-day or next-morning delivery. Volume has scaled 19.1% since 2024A without new space.

Near-term expansion - tri-state to regional markets

Licensed and credentialing-ready in four further Northeast states. IPA partnerships and low-capex draw sites (under $60K each) allow entry within a quarter; conservative suburban penetration assumptions are used in management's internal plan.

Long-term expansion - high-growth target states

High-growth southern and western states with favourable commercial reimbursement and senior populations growing above 3% annually. The molecular and toxicology menu is well suited to chronic-care and senior-living markets; entry would use overnight logistics before any satellite laboratory.



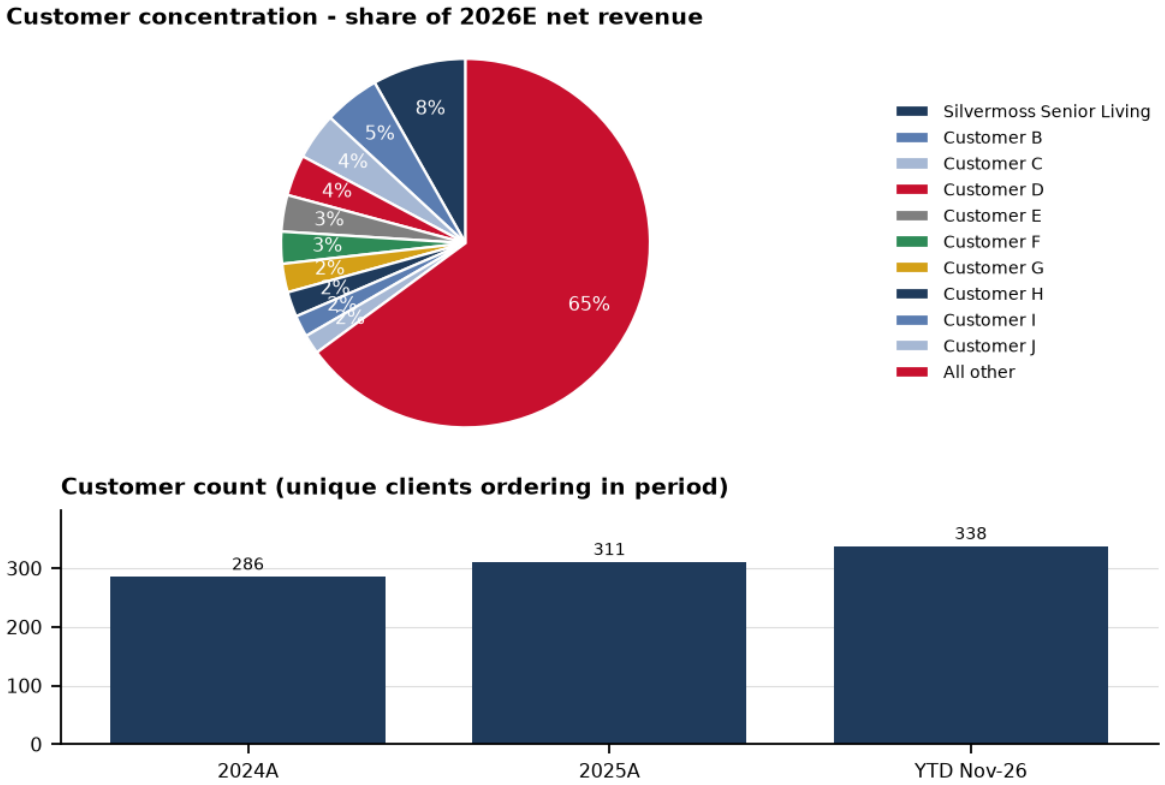
Current platform: States A, B, C; Regional growth: four licensed Northeast states; Target markets: three southern / western states

12-24 hrs	99.6%
Routine turnaround in all current states	First-pass reporting accuracy

- Map attribution: schematic, not to scale (synthetic).
- No geographic expansion revenue is included in the 2027E-2028E projections.

Source: Company management.

Customer Trends



Client type	% of 2026E net revenue
Physician practices & IPAs	57%
Behavioural-health providers	12%
Urgent-care clinics	13%
Senior-living communities	8%
Other (occupational, clinical trials)	10%

- 338 unique clients ordered in the eleven months to November 2026, up from 311 in 2025A and 286 in 2024A.
- The largest client, Silvermoss Senior Living, represents 8.2% of 2026E net revenue ($1.2M); no other client exceeds 4.9% and the top ten together account for 35.1%.
- Net logo retention of 103%: existing clients grew their spend by more than the revenue lost from departing clients.
- Churn is low and largely deliberate: four accounts were exited in 2025 for payment behaviour, representing under 0.5% of revenue.
- Commercial focus is on multi-site physician groups, IPAs and senior-living networks, which order daily, prefer bundled panels and are served by existing courier routes.

(1) Customer data as of 30 November 2026. (2) Net logo retention counts clients above $5,000 of annual billings. Source: Company management; QoE report (QoE Advisor) through Aug-26; Falkmere analysis.

Historical and Projected Income Statement and Commentary

$ in actual	2024A	2025A	2026E	2027E	2028E
Total accessions	85,204	92,761	101,458	113,147	136,217
YoY % growth	*n/a*	*8.9%*	*9.4%*	*11.5%*	*20.4%*
Implied reimbursement per accession	*$149.19*	*$148.31*	*$147.38*	*$143.53*	*$139.08*
Net operating revenue	**12,711,900**	**13,757,400**	**14,953,118**	**16,239,659**	**18,945,041**
YoY % growth	*n/a*	*8.2%*	*8.7%*	*8.6%*	*16.7%*
Operating expenses					
Medical supplies	317,200	343,800	430,923	414,111	483,099
Lab supplies	1,073,300	1,164,300	1,264,791	1,403,107	1,636,852
Drugs	114,800	123,600	100,193	144,533	168,611
Pharmacy	75,600	82,400	65,090	94,190	109,881
Total drugs & medical supplies	**1,580,900**	**1,714,100**	**1,860,997**	**2,055,941**	**2,398,442**
Salaries & wages	3,317,600	3,509,800	3,716,856	4,327,259	4,610,460
Contract labor	187,900	205,500	249,542	241,971	282,281
Sales rep expense	636,600	684,700	747,121	873,224	1,116,104
Other personnel costs	88,400	95,900	74,792	78,468	81,827
Total employee salaries & wages	**4,230,500**	**4,495,900**	**4,788,310**	**5,520,921**	**6,090,672**
Laboratory fees	729,800	792,200	863,135	948,396	1,106,390
Billing & collection fees	448,300	476,000	581,873	578,132	674,443
Total employee taxes & benefits	522,000	549,300	592,904	682,998	753,482
Total occupancy costs	710,400	768,100	806,807	825,178	860,511
All other operating expenses	1,996,400	2,143,600	2,237,575	2,357,629	2,594,093
Total operating expenses	**10,218,300**	**10,939,200**	**11,731,601**	**12,969,195**	**14,478,034**
% of revenue	*80.4%*	*79.5%*	*78.5%*	*79.9%*	*76.4%*
EBITDA, as adjusted	**2,475,500**	**2,800,700**	**3,207,267**	**3,255,118**	**4,451,005**
YoY % growth	*n/a*	*13.1%*	*14.5%*	*1.5%*	*36.7%*
% margin	*19.5%*	*20.4%*	*21.4%*	*20.0%*	*23.5%*

Commentary

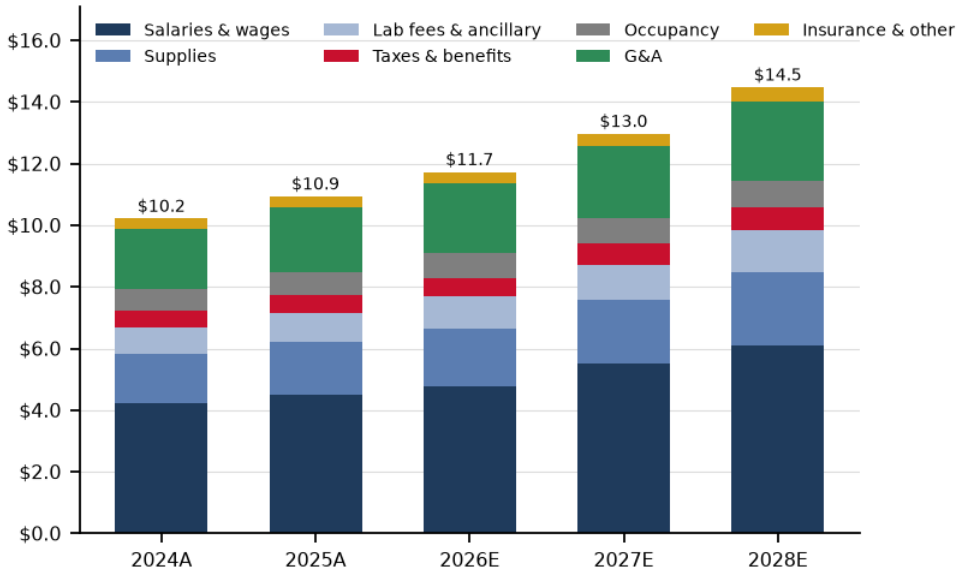
- Net operating revenue grows at a 12.6% CAGR from $15.0M in 2026E to $18.9M in 2028E, after 8.2% and 8.7% growth in 2025A and 2026E.
- Accessions of 85,204 in 2024A rise to 101,458 in 2026E and 136,217 in 2028E; the 2028E step reflects the first full year of the two representatives hired in March 2027 (31,820 accessions) and the Silvermoss contract (14,685).
- Growth is volume-driven: operating expenses fall from 80.4% of revenue in 2024A to 76.4% in 2028E as occupancy, IT and administrative costs are absorbed.
- 2027E margin dips to 20.0% because salaries and wages step up $0.6M for the Laboratory Manager, Shift Supervisor, IT Lead, couriers and accessioners, partly offset by the Medical Director transition.
- Management's plan assumes 510 average daily accessions by 2028E, 98% of the current single-shift ceiling; the capacity build-out is therefore timed for 2027-28.

(1) 2024A-25A and Jan-Aug 2026 per QoE report; Sep-Nov 2026 management-reported; Dec-26 onward projections. Other opex = ancillary, other medical, insurance, G&A. Source: Company management; QoE report (QoE Advisor) through Aug-26; Falkmere analysis.

Revenue Build and Operating Expense Profile

Revenue is built bottom-up from accessions per working day and net revenue per accession for each of the three engines

$ in actual	2024A	2025A	2026E	2027E	2028E
Existing sales team					
Accessions per workday	316.7	324.7	333.5	333.3	336.0
Workdays	269	270	268	262	267
Accessions	85,204	87,676	89,386	87,313	89,712
$ per accession	$149.19	$151.17	$153.51	$154.43	$154.43
Adj. net revenue	12,711,900	13,253,985	13,721,774	13,483,491	13,853,961
Senior-living contract (Silvermoss)					
Accessions per workday	0.0	18.8	45.0	52.4	55.0
Accessions	0	5,085	12,072	13,724	14,685
$ per accession	$0.00	$99.00	$102.00	$102.00	$104.00
Adj. net revenue	0	503,415	1,231,344	1,399,848	1,527,240
New sales team (from Mar-27)					
Accessions per workday	0.0	0.0	0.0	54.8	119.2
Workdays	0	0	0	221	267
Accessions	0	0	0	12,110	31,820
$ per accession	$0.00	$0.00	$0.00	$112.00	$112.00
Adj. net revenue	0	0	0	1,356,320	3,563,840
Total adj. net revenue	**12,711,900**	**13,757,400**	**14,953,118**	**16,239,659**	**18,945,041**
Total accessions	85,204	92,761	101,458	113,147	136,217
Assumed daily accessions	316.7	343.6	378.6	431.9	510.2
Max daily accessions (single shift)	520	520	520	520	520
% capacity	*60.9%*	*66.1%*	*72.8%*	*83.0%*	*98.1%*



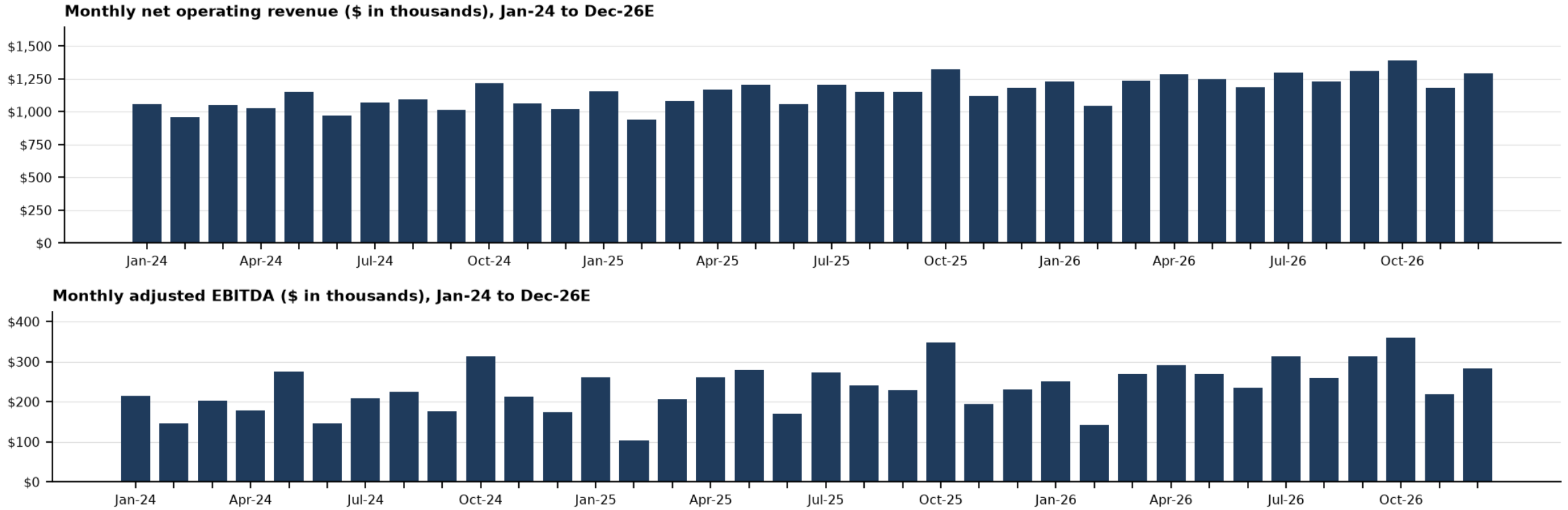
Operating expenses by category ($ in millions): salaries & wages, drugs & medical supplies, laboratory fees & ancillary services, employee taxes & benefits, occupancy, G&A (incl. billing & collection), insurance & other medical costs

- Employee salaries and wages are the largest cost at 32.0% of 2026E revenue; drugs and medical supplies (12.4%) and laboratory fees for send-outs (5.8%) are the principal variable costs.
- Total operating expenses grow from $11.7M in 2026E to $14.5M in 2028E, 11.1% annually, versus 12.6% revenue growth.
- Occupancy ($0.8M in 2026E) and G&A are largely fixed; fixed-cost projections use the trailing twelve-month average grown at a monthly inflation input.

Source: Company management projections (Revenue Build; Historical & Projected P&L).

Monthly Revenue and Adjusted EBITDA, 2024-2026

Thirty-six months of net operating revenue and adjusted EBITDA: LTM Nov-26 revenue of $14.8M and adjusted EBITDA of $3.2M

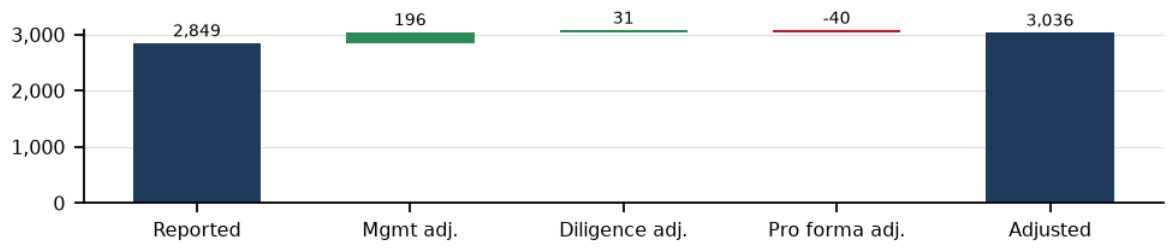


- Monthly revenue ranged from $944K to $1,394K over the period; October is the seasonal peak and February the trough, driven by working days (268 in 2026E) and respiratory-season molecular volume.
- Jan-Aug 2026 per the QoE report ($9.8M revenue, $2.0M adjusted EBITDA); Sep-Nov 2026 management-reported; Dec-26 projected at $1,291K.

Source: Company management; QoE report (QoE Advisor) through Aug-26; Falkmere analysis.

EBITDA Adjustments

$ in thousands	2024A	2025A	YTD Aug-26	LTM Aug-26
EBITDA, reported	**2,325.5**	**2,604.3**	**1,925.0**	**2,848.7**
Management adjustments				
(1) Owner compensation normalisation	186.0	192.0	132.0	196.0
Total management adjustments	**186.0**	**192.0**	**132.0**	**196.0**
EBITDA, management adjusted	**2,511.5**	**2,796.3**	**2,057.0**	**3,044.7**
Diligence adjustments				
(2) Reclassification of non-patient revenues	(16.5)	(18.4)	(8.6)	(13.1)
(3) Net revenue hindsight / accrual analysis	41.2	36.8	27.4	44.9
(4) Removal of non-recurring and personal expenses	58.3	64.1	39.7	61.2
(5) Occupancy cost normalisation	(22.0)	(22.0)	(14.7)	(22.0)
(6) Medical supply expense normalisation	(9.4)	7.6	5.1	9.8
(7) Insurance normalisation	3.8	4.1	2.9	4.4
(8) Lab equipment lease normalisation	(11.6)	(12.2)	(8.4)	(12.6)
(9) Go-forward sales representative expense	(24.0)	(26.5)	(18.0)	(27.0)
(10) Other operating expense normalisation	14.7	9.9	6.3	11.4
(11) Restating payroll to payroll registers	(31.2)	(28.4)	(17.9)	(26.7)
(12) Normalised contracted provider compensation	18.0	18.0	12.0	18.0
(13) Payroll cash-to-accrual	(6.9)	4.2	(9.8)	(7.1)
(14) Accounts payable cash-to-accrual	**(12.4)**	**8.7**	**(15.3)**	**(10.2)**
Total diligence adjustments	**2.0**	**45.9**	**0.7**	**31.0**
EBITDA, diligence adjusted	2,513.5	2,842.2	2,057.7	3,075.7
Pro forma adjustments				
(15) Reference lab utilisation normalisation	**(38.0)**	**(41.5)**	**(27.0)**	**(40.1)**
Total pro forma adjustments	**(38.0)**	**(41.5)**	**(27.0)**	**(40.1)**
Total adjusted EBITDA	2,475.5	2,800.7	2,030.7	3,035.6



LTM Aug-26 bridge from reported to adjusted EBITDA ($ in thousands)

Commentary

- (1) Owner compensation normalised to a market salary for the CEO role; the difference between distributions taken as compensation and the market rate is added back.
- (2) Interest and other non-patient income recorded in service revenue reclassified below EBITDA.
- (3) Net revenue restated on a hindsight basis using cash collected on each month's claims; the Company reports on a cash basis.
- (4) Removal of non-recurring legal fees, a one-time LIS migration cost and personal expenses run through the business.
- (5) Occupancy normalised to the current lease rate, reversing a rent holiday and property-tax true-ups.
- (6) Supply cost per accession normalised for bulk reagent purchases straddling period ends.
- (7) Insurance premiums accrued evenly across the policy year.
- (8) Equipment leases treated consistently as operating expense across periods.
- (9) Sales representative expense restated to the go-forward commission plan; a vendor overpayment on marketing services removed.
- (10) Credentialing, licensing and software subscriptions allocated to the periods they cover.
- (11) Payroll restated to the payroll registers to correct account misallocations.
- (12) Contracted pathologist compensation normalised to executed agreements.
- (13) Accrued payroll converted from cash to accrual basis.
- (14) Accounts payable converted from cash to accrual basis.
- (15) Outsourced reference-laboratory fees normalised to the post-relocation utilisation rate.

(1) Adjustments per the QoE report prepared by QoE Advisor (independent accounting firm) covering Jan-24 to Aug-26; cash-basis reporting. Source: Company management; QoE report (QoE Advisor) through Aug-26; Falkmere analysis.

Balance Sheet - Reported

$ in thousands	Dec-24	Dec-25	Nov-26
Assets			
Current assets			
Cash & equivalents	1,412.4	1,687.9	1,954.2
Inventories	318.2	341.6	362.9
Other current assets	96.3	104.8	118.1
Due from affiliates	42.0	38.5	35.0
Total current assets	**1,868.9**	**2,172.8**	**2,470.2**
Fixed assets			
Leasehold improvements	1,860.4	1,912.7	1,988.3
Furniture & fixtures	214.1	226.5	231.8
Equipment	2,406.9	2,571.3	2,689.6
Accumulated depreciation	(2,918.6)	(3,180.1)	(3,352.5)
Net fixed assets	**1,562.8**	**1,530.4**	**1,557.2**
Other assets			
Security deposits	58.0	58.0	61.2
Total other assets	**58.0**	**58.0**	**61.2**
Total assets	**3,489.7**	**3,761.2**	**4,088.6**
Liabilities and members' equity			
Current liabilities			
Accrued salaries & benefits	286.4	301.2	322.6
Current portion of long-term debt	118.0	112.0	104.0
Credit cards	41.7	37.2	44.9
Total current liabilities	**446.1**	**450.4**	**471.5**
Long-term debt, net of current portion	402.5	290.5	196.4
Total liabilities	**848.6**	**740.9**	**667.9**
Equity			
Retained earnings	2,157.1	2,551.3	2,886.2
Members' distributions	(1,980.0)	(2,310.0)	(2,450.0)
Net income	2,214.0	2,529.0	2,734.5
Members' equity	250.0	250.0	250.0
Total equity	**2,641.1**	**3,020.3**	**3,420.7**
Total liabilities and members' equity	**3,489.7**	**3,761.2**	**4,088.6**

(1) Unaudited, cash basis. Totals may not sum due to rounding. Source: Company management.

Notes to the reported balance sheet

- Cash-basis, unaudited balance sheets as reported by management at 31 December 2024, 31 December 2025 and 30 November 2026; receivables are not recorded under the cash basis and are addressed in the QoE net working capital analysis.
- The Company carries only 300.4 thousand of equipment-related term debt at November 2026, with 2026E interest expense of $35K; the business is expected to be delivered cash-free, debt-free.
- Members' equity reflects the LLC structure: net income of 2,734.5 thousand for the eleven months to November 2026 (after $172K of depreciation) and distributions of 2,450.0 thousand.
- Leasehold improvements and equipment are stated at cost; the second-floor build-out and adjacent space are not yet capitalised.

Key Takeaways

Diversified testing portfolio

Four service lines and 410 tests across routine, molecular, pathology and toxicology, serving five client types with no client above 8.2% of revenue.

Scalable infrastructure

73% single-shift utilisation, a pre-approved build-out and automation that let capacity double within the existing lease.

Strong reimbursement visibility

In-network with government programmes and seven commercial payers; collection rate stable at 42.5% of gross charges; net revenue per accession of $147.38 in 2026E.

Experienced leadership and staff

Six leaders averaging 13 years of tenure, 41 laboratory professionals and a planned Medical Director transition.

Platform positioned for growth

$15.0M of 2026E revenue and $3.2M of adjusted EBITDA growing to $18.9M and $4.5M by 2028E on contracted and funded initiatives, with geographic expansion as further upside.



Transaction Overview, Process and Contacts

Falkmere Advisory has been engaged as exclusive financial advisor for the sale of 100% of the membership interests of the Company

- The members of Orivane Clinical Laboratories, LLC are seeking a partner to acquire 100% of the membership interests; the Founder is open to a meaningful equity rollover and a multi-year transition. The Company is to be delivered cash-free and debt-free with a normalised level of net working capital to be agreed in definitive documentation.
- A two-stage process: indicative, non-binding offers based on this Memorandum, followed by management presentations, data-room access and the QoE report (through Aug-26) for a limited number of parties. Indicative offers should state enterprise value, the basis of valuation, financing sources, key assumptions, diligence requirements, approvals and timing to signing.
- All communications must be directed to Falkmere Advisory; under no circumstances should the Company, its employees, clients, payers or suppliers be contacted directly.

Dec-26	Week of 25 Jan-27	Feb-27	Mid Mar-27	Apr-27	Q2-27
CIM distributed	Indicative offers due	Management presentations, data room, site visits	Final binding offers with marked-up purchase agreement	Exclusivity, confirmatory diligence, signing	Closing (subject to licence and payer notifications)

Falkmere Advisory - contacts (Headquarters, State D; www.falkmere.example)

Jonathan Falkmere	Managing Director	+1 (555) 010-0101	jonathan.falkmere@falkmere.example
Elena Vasquez-Reid	Director	+1 (555) 010-0114	elena.vasquez-reid@falkmere.example
Tobias Grant	Vice President	+1 (555) 010-0127	tobias.grant@falkmere.example
Hannah Okoro	Associate	+1 (555) 010-0139	hannah.okoro@falkmere.example

Source: Falkmere Advisory.

Appendix - Quality of Earnings Summary and Glossary

Quality of Earnings summary

$ in thousands	2024A	2025A	YTD Aug-26	LTM Aug-26
Net operating revenue ($M)	$12.7M	$13.8M	$9.8M	$14.6M
EBITDA, reported	2,325.5	2,604.3	1,925.0	2,848.7
Total adjustments (15 items)	150.0	196.4	105.7	186.9
Total adjusted EBITDA	**2,475.5**	**2,800.7**	**2,030.7**	**3,035.6**
Adjusted EBITDA margin	*19.5%*	*20.4%*	*20.8%*	*20.9%*

- Scope: 32 months (Jan-24 to Aug-26) of cash-basis financial statements, payroll registers, bank statements, the general ledger and payer remittances, prepared by QoE Advisor, an independent accounting firm.
- Key findings: revenue substantiated to remittances and hindsight collections; adjustments principally relate to owner compensation, cash-to-accrual timing and non-recurring costs; no adverse trends in reimbursement or denials were identified.
- Net working capital: a normalised level will be proposed based on the accrual-basis analysis and agreed in definitive documentation.

Glossary

Term	Definition
Accession	A specimen or set of specimens received and logged from one patient encounter; the Company's unit of volume.
Net revenue per accession	Net operating revenue divided by accessions; the blended reimbursement realised per encounter.
Gross charges / collection rate	Billed charges at list price; collection rate is net operating revenue as a percentage of gross charges.
Procedure code	Billing code under which each test is billed to payers.
LIS / EMR	Laboratory information system; electronic medical record used by clients.
PSC	Patient service centre: a company-staffed draw site.
TAT	Turnaround time from specimen receipt to result release.
IPA	Independent physician association contracting on behalf of member practices.
QoE	Quality of Earnings report prepared by an independent accounting advisor.
A / E / P	Actual / estimate / projection; LTM = last twelve months; YTD = year to date.

Source: Company management; QoE report (QoE Advisor) through Aug-26; Falkmere analysis.